

Individual Contribution Statement

Project: NeuroTriage AI | Student: Saksha Hegde

1. Individual Role

I acted as the **Product Owner and Project Manager** for the NeuroTriage AI project. I was responsible for defining the problem, product direction, scope, workflow, priorities, acceptance criteria, and overall delivery. I used AI tools as part of a structured delivery team: **Claude was used in the role of a Software Lead Developer and Architect**, while **ChatGPT was used as a Technical Consultant** for technical reasoning, product/technical review, problem solving, and validation. I retained ownership of all product and delivery decisions.

2. Key Contributions

- **Product strategy and problem framing:** Defined the clinical workflow problem, target user, product vision, JTBD, first wedge, MVP scope, and the principle of prioritizing suspected ICH rather than replacing the radiologist.
- **Requirements and UX:** Defined the worklist, priority model, reading experience, confidence display, AI overlay, human-in-the-loop Confirm/Override flow, CT windowing, and trust-oriented interaction design.
- **Architecture and technical direction:** Worked with Claude as the software lead/architect to shape the backend/frontend structure, repositories, workflow, API behaviour, simulated AI interface, test strategy, and implementation approach.
- **Technical consultation and review:** Used ChatGPT as a technical consultant to challenge design choices, reason through implementation options, identify risks, improve requirements, and review outputs during development.
- **Prototype delivery:** Guided implementation of the functional end-to-end prototype using real anonymized CT studies, including staged study arrival, automatic reprioritization, calibrated predictions, image review, Jump to Finding, AI overlays, feedback capture, and Reset Demo.
- **Data and image validation:** Personally reviewed the converted CT images and validated ICH visibility, overlay coordinates, slice selection, windowing, and laterality rather than relying solely on automated outputs.
- **QA and acceptance:** Led the final end-to-end demonstration QA, including the complete 9-step workflow, Confirm/Override behaviour, worklist reprioritization, overlay alignment, Reset Demo, and DICOM/image integrity checks.
- **Documentation and submission:** Directed the final report, prototype documentation, AI collaboration documentation, supporting PM artifacts, and final submission package.

3. AI Collaboration Model

Product Owner / Manager — Me: Problem definition, product strategy, requirements, prioritization, scope, acceptance criteria, clinical workflow interpretation, risk decisions, validation, and final approval.

Software Lead Developer / Architect — Claude: Architecture, implementation planning, coding, debugging, automated testing, browser-based verification, and iterative technical execution under my direction.

Technical Consultant — ChatGPT: Technical reasoning, design alternatives, requirement refinement, review, troubleshooting, documentation support, and independent challenge of implementation decisions.

4. Individual Judgment and Corrections

I did not treat AI-generated output as automatically correct. I reviewed and challenged recommendations, refined the scope, separated simulated AI inference from claims about a clinical model, validated real CT findings and overlay placement, checked laterality, and ensured that the final workflow preserved mandatory human oversight. The final prototype and submission therefore reflect product and engineering decisions that were reviewed and accepted by me.

5. Reflection

AI significantly accelerated the project by allowing me to work with a virtual software development team and technical consultant while I remained accountable as Product Owner and Manager. Claude enabled rapid architectural and implementation execution, while ChatGPT provided an independent technical sounding board. The experience also reinforced that AI can accelerate delivery, but it does not replace product judgment: deciding what to build, what not to build, how much to trust an AI output, how to handle clinical risk, and when an implementation is actually acceptable remained my responsibility.

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Date
09 August 2026